EXHIBIT 5

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January 2, 2026

Chip Wagar, Esq.
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1401 W. Causeway Approach
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RE: Katherine Moller o/b/o Harmony Moller (D) v Martian Sales, Inc., et al; US District Court, Eastern District of Louisiana; Civil Action NO. 24-0228-WBV-DPC

Dear Mr. Wagar:

This is a follow-up to my initial report dated October 19, 2025. You have asked that I opine on the reports of Drs. Allen L. Dollar, Dr. Amber Wang, and Dr. Winston Gandy Jr.

Since the opinions of these physicians are based upon toxicology and the post-mortem macroscopic and microscopic findings at autopsy, I would first like to summarize what I believe to be the pertinent findings:

1) It is known that Ms. Moller had a "high takeoff" of the left coronary artery.
2) Promethazine was found at a concentration of 18 ng/cc.
3) Diphenhydramine was found at a concentration of 64 ng/cc
4) Microscopic pathology noted "section of ventricular septum has normal arrangement of myocytes within the compact myocardium. Myocytes are hypertrophic and there are scattered hypereosinophilic myocytes." There were findings noted in the macroscopic pathology which I will address below.

My responses are as follows:

1) The "high takeoff of the left coronary artery" is not associated with sudden cardiac death (as an aside, the pathology report makes no mention of an "interarterial or intramural course" of the left coronary artery). The known anomaly associated with sudden death is that in which the left coronary artery takes its origin from the right coronary cusp and in doing so courses between the aorta and the main pulmonary artery, which is not the case here. Furthermore, those coronary anomalies which are associated with sudden death do so during periods of exertion.
2) Promethazine and diphenhydramine can prolong the QT-interval. There is a growing body of evidence that molecules present in Kratom prolong the QT-interval.
3) The hallmark of hypertrophic cardiomyopathy is the presence of myocardial disarray. A "normal arrangement of myocytes" is not compatible with the presence of hypertrophic cardiomyopathy, nor is the presence of "eosinophilic myocytes".

It is standard of care to caution patients about polypharmacy which could have adverse synergistic effects. This is certainly true when considering the potential for QT

prolongation. Even if the concentration of an individual agent is considered not to be dangerous, when multiple agents are taken together, there is a risk for dangerous QT prolongation which can result in polymorphic ventricular tachycardia.

The pathology report is not consistent with hypertrophic cardiomyopathy (HCM).

a) The left ventricle was reported as "mildly dilated". The left ventricular cavity in patients with HCM is small, and certainly not "dilated".
b) The heart's weight was reported as only 10 grams above the reference range.
c) There is a growing body of evidence linking Kratom use to hypertension, which could lead to hypertensive heart disease (i.e. left ventricular hypertrophy)[1]. It is certainly possible that the left ventricular hypertrophy noted at autopsy was a manifestation of Kratom exposure.
d) Myocardial disarray, the ***hallmark*** of hypertrophic cardiomyopathy, was ***not*** found in the microscopic pathology. This finding is associated with dangerous arrhythmias[2,3]. The absence of this finding precludes the diagnosis of HCM.
e) Those risk factors for sudden death in patients with a history of HCM about which we have knowledge (left ventricular wall thickness $\geq$ 30 mm, family history of sudden cardiac death, left ventricular apical aneurysm, unexplained syncope) are decidedly absent.

The combination of promethazine, diphenhydramine, and Kratom can synergistically lead to QT interval prolongation and consequent polymorphic ventricular tachycardia.

Conclusions:

1. The high ostia take-off of the left coronary artery is a benign finding which was not a cause of her sudden death.
2. The pathology report, both macroscopic and microscopic, is inconsistent with a diagnosis of HCM.
3. In my opinion, more likely than not, Harmony Moller died from a ventricular arrhythmia induced by consumption of Kratom, especially in combination with two other agents known to prolong the QT interval.

The conclusions reached in this report are based on my education, training, and experience, and are all within reasonable medical probability. This report is based on the material I have been asked to review as of January 2, 2026. I am being compensated at a rate of $700/hr for my consultation in this case. I reserve the right to amend this report should further information become available.

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References:

1) Chicagi, F. et. al. Cardiovascular health in kratom users; a narrative review. Journal of Addictive Diseases 2024, 42,4: 313-25.
2) Varnava, A.M. et. al. Relation between myocyte disarray and outcome in hypertrophic cardiomyopathy. Am J Cardiol 2001 Aug 1;88(3):275-9.
3) Ariga, R. et. al/ Identification of Myocardial Disarray in Patients With Hypertrophic Cardiomyopathy and Ventricular Arrhythmias. J Am Coll Cardiol 2019 May 28;73(20):2493-2502